Royal Devon University Healthcare NHS Foundation Trust

Clinical Guideline for:
Antenatal care for pregnant women and referral to consultant care
Eastern Services

Summary

This guideline summarises the information that all pregnant women should be provided with to support and enable them to make informed decisions regarding their care.

Key Points

The essential elements of this guideline are:

- To provide a schedule of Antenatal appointments for all pregnant women
- To provide a recommendation of care that should be delivered by midwives at each antenatal visit
- To establish a clear indicator of referral pathways for women with obstetric, medical or social risk factors in pregnancy.
- To provide lifestyle advice that should be given to pregnant women to optimise health in pregnancy
- To offer health promotion advice in pregnancy

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1.0 INTRODUCTION

1.1 All pregnant women should be provided with evidenced-based information to support and enable them to make informed decisions regarding their care. Women's choices should be integral to the decision making process. Information should be provided on location of antenatal care and who will undertake their care.

1.2 All care provided should be documented in the electronic patient record. A schedule of appointments should be provided at the booking appointment.

1.3 Should a woman develop risk factors in pregnancy a referral to the team's obstetric consultant must be completed.

1.4 The schedule of antenatal appointments below represents the recommended number of antenatal care appointments for women who are healthy and whose pregnancies remain uncomplicated in the antenatal period: ten appointments for nulliparous women and seven for parous women.

1.5 For women who are more vulnerable who may have specific social needs increased antenatal care should be considered and home visits offered. For women who do not speak English an interpreter should be utilised at each antenatal contact either via language line or in person (CEMACH 2003-5).

1.6 Referrals to maternity services may be received from a variety of sources in order to promote choice and accessibility of maternity care. The women may self-refer directly to a midwife or choose to access services via their GP.

1.7 For women who do not attend for appointments either in the hospital or in the community follow the Guidelines for follow up of non-attendees for antenatal care.

1.8 Throughout the entire antenatal period, healthcare providers should remain alert to signs or symptoms of conditions which affect the health of the mother and baby. If deviations from the norm occur then an Obstetric Consultant referral should be made.

1.9 It is the midwife's responsibility to contact a consultant directly if there are urgent concerns or a conflict of opinion with the Obstetric Team regarding the care of women and the baby.

2.0 BOOKING APPOINTMENT(S) (7-12 WEEKS)

2.1 The first appointment needs to be early in pregnancy (7-12 weeks). Due to the large volume of information provided in early pregnancy, two appointments may be required. One of these appointments may be as part of the Maternity Services Welcome appointment clinic led by the Maternity Support workers.

2.2 At this appointment:

- Measure and record blood pressure
- Test urine for protein
- Measure BMI
- Perform Carbon Monoxide (CO) test, ask about smoking status and refer (opt out) if smoking or quit in last 2 week.
- Identify women who have FGM
- Ensure the electronic patient record is completed and women are provided with a maternity passport and personalisation journal.

- Send all appropriate referrals
- Discuss planned pattern of pregnancy care
- Ensure all contact details are correctly documented
- Make appointment for first trimester screening clinic and provide National Screening Leaflet.
- Discuss routine screening including blood group and RhD status and offer screening for anaemia, red-cell antibodies, hepatitis B virus, HIV, syphilis and asymptomatic bacteriuria. Screening for sickle cell and thalassaemia (FBC and FOQ) must be ordered by the midwife via the electronic patient record at booking. The screening test should be taken at the next Maternity Services Welcome clinic, ideally before 10 weeks gestation. The result must be checked promptly by the named midwife or MSW who takes the bloods. Results will be available to view in each community results pool and these are checked daily.
- Offer first trimester combined (nuchal translucency and serum screen) screening for Down's, Edwards' and Patau's syndrome
- Carry out a risk assessment (see **Section 3** below)
- Assess for risk factors for pre-eclampsia (see table in Hypertension Guideline) and advise aspirin 150mg OD from 12 weeks if indicated.
- Ensure venous thromoboembolism (VTE) risk assessment undertaken and action taken as necessary.
- Identify women who have had a past or present mental health illness by using the Mental Health Prediction & Detection tool – refer as necessary.

3.0 RISK ASSESSMENT

3.1 All community midwives are responsible for completing a full booking risk assessment detailing maternal risk factors relating to previous obstetric history, maternal medical history, family history, psychiatric history, anaesthetic history and general health and wellbeing. This history should be completed in the electronic patient record ideally between 7 and 12 weeks of pregnancy.

3.2 If risk factors are identified, the midwife should complete a referral to the Team's Obstetric Consultant. Women will be allocated to the named consultant for their community team unless they have specific requirements such as multiple pregnancy. All consultant referrals are reviewed regularly.

3.3 All women will undergo a Risk Assessment for Preterm Birth and Fetal Growth Restriction. This will be commenced by the midwife at the booking appointment and completed by an FMAU midwife following FTSC. Any additional scans indicated will be ordered and information provided to the woman.

3.4 If at any point in the pregnancy the woman becomes higher risk then the midwife must transfer the woman to consultant led care via a referral through the electronic patient record. The lead professional must be documented in the electronic patient record clearly indicating midwife-led or consultant-led care.

3.5 If there is doubt regarding a woman's risk status then consultants are happy to discuss cases or see women in antenatal clinic as appropriate. Women may then be transferred back to the community for the remainder of their care if appropriate; this will be documented within the electronic patient record.

3.6 Higher risk women will have a pregnancy management plan formulated in antenatal clinic and documented in the electronic patient record. The midwife must ensure that she remains familiar with the plan for antenatal care and as the woman's named midwife.

Where a referral is made due to maternal fear/anxiety of childbirth, or a request for elective LSCS, it may be considered appropriate for the midwife to offer support by attending the Consultant or counselling referral appointment. Mothers with any of these risk factors would not normally be suitable for Midwife led unit or home birth.

3.7 A risk assessment must be undertaken at every antenatal contact, whether with a midwife or an obstetrician. This should be recorded in the Antenatal Care Pathway Review section of the electronic patient record.

3.8 If a woman discloses that they have had female genital mutilation (FGM) a datix needs to be completed. The midwife should discuss with the woman that a MASH referral will be made following birth if the child is female and the safeguarding team will add the alert to the FGM-IS unless already present. If other female children within the family are at risk please refer to the Safeguarding Children Policy.

4.0 LATE BOOKERS

4.1 For women who are referred to the maternity service after 12 weeks of pregnancy an urgent booking should be completed and an urgent referral made to the ultrasound scan department – this appointment should be classed as a FTS appointment to ensure the correct pathways are followed. The initial ultrasound/ dating scan should be completed within 2 weeks of the booking. If the midwife is unable to organise an urgent appointment or Ultrasound scan she should escalate the issue urgently to her line manager. All booking bloods, including sickle/thal (FOQ) and infectious disease screening, even if late in pregnancy, should be taken by the community midwife as soon as possible.

If an unbooked woman attends the RDE in labour, screening should be offered and sent to the lab for urgent testing, even if overnight. The screening coordinator midwife must be informed of this situation.

4.2 Women newly arrived to the UK who have not previously had a full medical examination should be advised on how to register with a GP. The midwife should then facilitate the woman to attend the GP's surgery for a full medical examination. The midwife needs to document her actions in the electronic patient record. All booking bloods, including sickle/thal (FOQ) and infectious disease screening, should be taken as soon as possible. An interpreter should be considered for any woman whose 1st language is not English.

If a woman books with RDE midwives after moving into the area from another UK Trust during pregnancy, it is the responsibility of the RDE to ensure she has been previously offered infectious disease and sickle/thalassaemia screening and results followed up. Lab results from the previous UK Trust must be uploaded to the media section of the electronic patient record and recorded as an external result on the Results Console.

If there are no results or the woman has moved from abroad, screening must be re-offered and documented on the electronic patient record.

5.0 SIXTEEN WEEKS GESTATION (16)

5.1 The next appointment should be scheduled at 16 weeks to:

- Review, discuss and document the results of all screening tests undertaken. If a woman DNA's this appointment her results must still be reviewed.
- Women who are rhesus negative will need their cell free DNA result reviewing and discussing. Those with a rhesus negative baby will require no further intervention during pregnancy.
- Rhesus negative mothers with a rhesus positive baby will be offered prophylactic anti D. The result will be independent to each pregnancy. Offer these ladies an explanation leaflet regarding their RhD status.
- Reassess planned pattern of care for the pregnancy and identify women who need additional care.
- Investigate a haemoglobin level of less than 11 g/dL and consider iron supplementation if indicated
- Measure blood pressure and test urine for proteinuria.
- Count the Kicks leaflet should be given out and discussion about fetal movements in pregnancy
- Discuss planned place of birth and the options available to the woman.

6.0 EIGHTEEN TO TWENTY WEEKS GESTATION (18–20)

6.1 At 18 - 20 weeks, if the woman chooses, an ultrasound scan should be performed for the detection of structural anomalies. For a woman whose placenta is found to extend across the internal cervical os at this time, another scan at 32 weeks should be offered.

6.2 For women that decline screening, after FTSC and before 20 weeks gestation the screening coordinator will make contact with the woman to re-offer screening and ensure she has made a fully informed decision. If she wishes to discuss further, a face to face meeting will be arranged with the screening coordinator. If the woman decides to be screened the screening coordinator will arrange this.

7.0 TWENTY- FIVE WEEKS GESTATION (25)

7.1 At 25 weeks of gestation, another appointment should be scheduled for nulliparous women and women who are RhD negative. At this appointment:

- Measure blood pressure and test urine for proteinuria
- For Rh negative women who have been appropriately screened and shown to be carrying a Rh D positive fetus discuss Anti D and order if consent obtained.
- Rhesus negative women carrying a rhesus negative fetus will not require anti D.
- Give information, with an opportunity to discuss issues and ask questions; offer verbal information supported by antenatal classes and written or digital information. Discuss Vaginal Birth after Caesarean (VBAC) leaflet if woman has had a previous caesarean section.

8.0 TWENTY- EIGHT WEEKS GESTATION (28)

8.1 The next appointment for all pregnant women should occur at 28 weeks. At this appointment:

- Offer a second screening for anaemia and atypical red-cell antibodies
- Investigate a haemoglobin level of less than 10.5 g/dl and consider iron supplementation, if indicated
- Offer anti-D to rhesus-negative women who have received appropriate screening and are carrying a rhesus positive fetus. If fetal blood grouping is unknown the woman should be offered prophylactic anti-D. Ensure the appropriate consent form is completed and rationale understood.
- Reoffer infectious disease screen if declined at FTSC
- Repeat VTE risk assessment and take action if necessary.
- Measure blood pressure and test urine for proteinuria.
- Measure and record the woman's blood pressure at every routine face to-face antenatal appointment using a device validated for use in pregnancy, and following the recommendations on measuring blood pressure in the NICE guideline on hypertension in adults
- https://www.england.nhs.uk/long-read/recommendations-for-digital-blood-pressure-monitoring-in-maternity-services/
- Measure and plot symphysis–fundal height. In low risk women this needs to be done by 28+6. If a woman is having serial growth scans commencing at 28 weeks then fundal height measurement is not required.
- Give information, with an opportunity to discuss issues and ask questions; offer verbal information supported by antenatal classes and written or digital information.

9.0 THIRTY- ONE WEEKS GESTATION (31)

9.1 Nulliparous women should have an appointment scheduled at 31 weeks to:

- Measure blood pressure and test urine for proteinuria
- Measure and plot symphysis–fundal height if the woman is NOT having serial growth scans in pregnancy.
- Give information, with an opportunity to discuss issues and ask questions; offer verbal information supported by antenatal classes and written or digital information. Signpost women to where they can find relevant patient information leaflets digitally, providing paper copies at the woman's request.
- Review, discuss and document the results of screening tests undertaken at 28 weeks; reassess planned pattern of care for the pregnancy and identify women who need additional care (referral to consultant Obstetrician guidelines).

10.0 THIRTY- FOUR WEEKS GESTATION (34)

10.1 At 34 weeks, all pregnant women should be seen in order to:

- Measure blood pressure and test urine for proteinuria
- Measure and plot symphysis–fundal height if the woman is NOT having serial growth scans during pregnancy.

- Give information, with an opportunity to discuss issues and ask questions; offer verbal information supported by antenatal classes and written or digital information.
- Review, discuss and document the results of screening tests undertaken at 28 weeks; reassess planned pattern of care for the pregnancy and identify women who need additional care.

11.0 THIRTY- SIX WEEKS GESTATION (36)

11.1 At 36 weeks, all pregnant women should be seen to:

- Measure blood pressure and test urine for proteinuria
- Perform CO test on **all** pregnant women
- Measure and plot symphysis–fundal height if the woman is NOT having serial growth scans in pregnancy.
- Check position of baby refer for scan if unsure of position or baby thought to be breech
- For women whose babies are in the breech presentation, offer external cephalic version and book appointment via the fetal and maternal assessment unit. Signpost to leaflet 'Breech baby at the end of pregnancy'.
- Discuss feeding intention.. Complete checklist in pregnancy checklist if not completed previously.

11.2 Provide information, with an opportunity to discuss issues and ask questions; offer verbal information supported by antenatal classes and written or digital information.

11.3 Discuss preparation for labour and birth including the birth plan, recognising active labour and coping with pain.

11.4 Briefly discuss newborn screening and signpost them to their Screening Tests for You and Your Baby leaflet.

12.0 THIRTY- EIGHT WEEKS GESTATION (38)

12.1 Another appointment at 38 weeks will allow for:

- Measurement of blood pressure and urine testing for proteinuria
- Measurement and plotting of symphysis–fundal height if the woman is NOT receiving serial growth scans in pregnancy.
- Complete clinical pregnancy risk assessment and document on electronic patient record.
- Information giving, with an opportunity to discuss issues and ask questions
- Verbal information supported by antenatal classes and written or digital information.
- If induction of labour is planned for 39 weeks, a membrane sweep can be offered at this appointment.

13.0 FORTY WEEKS GESTATION (40)

13.1 For nulliparous women, an appointment at 40 weeks should be scheduled to:

- Measure blood pressure and test urine for proteinuria
- Measure and plot symphysis–fundal height if the woman is NOT having serial growth scans in pregnancy.
- A membrane sweep should be offered
- Options for management of prolonged pregnancy
- Give information, with an opportunity to discuss issues and ask questions; offer verbal information supported by antenatal classes and written or digital information.

14.0 FORTY- ONE WEEKS GESTATION (41)

14.1 For women who have not given birth by 41 weeks:

- A membrane sweep should be offered (multigravida and primigravida)
- Induction of labour should be offered
- Blood pressure should be measured and urine tested for proteinuria
- Symphysis–fundal height should be measured and plotted if the woman is NOT having serial growth scans in pregnancy.
- Information should be given, with an opportunity to discuss issues and ask questions; verbal information supported by written or digital information on Induction of Labour.

15.0 FROM FORTY-TWO WEEKS GESTATION (42)

15.1 Offer women who decline IOL increased monitoring – twice weekly CTG & US measurement of AFI. Refer to FMAU and consultant led care.

16.0 LIFESTYLE ADVICE

16.1 At the first (and second) antenatal appointment women should be provided with information to promote optimum health and wellbeing in pregnancy.

16.2 Pregnant women should be informed of their maternity rights and benefits.

16.3 The majority of women can be reassured that it is safe to continue working during pregnancy. Further information about possible occupational hazards during pregnancy is available from the Health and Safety Executive (www.hse.gov.uk).

16.4 A woman's occupation during pregnancy should be ascertained to identify those at increased risk through occupational exposure.

16.5 Pregnant women (and those intending to become pregnant) should be informed that dietary supplementation with folic acid, before conception and up to 12 weeks of gestation, reduces the risk of having a baby with neural tube defects (anencephaly, spina bifida). The recommended dose is 400 micrograms per day.

16.6 Few medicines have been established as safe to use in pregnancy. Prescription medicines should be used as little as possible during pregnancy and should be limited to circumstances where the benefit outweighs the risk. Few over the counter (OTC)

medicines have been established as being safe to take in pregnancy women should be advised to seek advice from a pharmacist.

16.7 Pregnant women should be informed that beginning or continuing a moderate course of exercise during pregnancy is not associated with adverse outcomes.

16.8 Pregnant women should be informed of the potential dangers of certain activities during pregnancy, for example, contact sports, high-impact sports and vigorous racquet sports that may involve the risk of abdominal trauma, falls or excessive joint stress, and scuba diving, which may result in fetal birth defects and fetal decompression disease.

16.9 Pregnant woman should be informed that sexual intercourse in pregnancy is not known to be associated with any adverse outcomes.

16.10 Inform women younger than 25 years about the high prevalence of Chlamydia and give details of local screening programme.

16.11 If you are aware of any lifestyle factors within the household that would put the baby at risk of Hep B infection, please refer to the Public Health Midwives, as the baby would be recommended to have a dose of Hep B vaccine within the 1st 24 hours of life.

17.0 ALCOHOL CONSUMPTION IN PREGNANCY

17.1 Pregnant women and women planning a pregnancy should be advised to avoid drinking alcohol in the first 3 months of pregnancy if possible because it may be associated with an increased risk of miscarriage. If women choose to drink alcohol during pregnancy they should be advised to drink no more than 1 to 2 UK units once or twice a week (1 unit equals half a pint of ordinary strength lager or beer, or one shot (25 ml) of spirits. One small (125 ml) glass of wine is equal to 1.5 UK units).

17.2 Although there is uncertainty regarding a safe level of alcohol consumption in pregnancy, at this low level there is no evidence of harm to the unborn baby.

17.3 Women should be informed that getting drunk or binge drinking during pregnancy (defined as more than 5 standard drinks or 7.5 UK units on a single occasion) may be harmful to the unborn baby.

18.0 SMOKING IN PREGNANCY

18.1 At the first contact with the woman, perform a Carbon Monoxide (CO) test, ask about her smoking status, provide written and verbal information about the risks of smoking to the unborn child and the hazards of exposure to secondhand smoke, advise her to quit. Refer to specialist smoke stop service for quitting support (this is an opt out referral). Address any concerns she and her partner or family may have about stopping smoking. Advise that cutting down does not benefit her or her baby.

18.2 Pregnant women should be informed about the specific risks of smoking during pregnancy (such as the increased risk of having a baby with low birth weight, stillbirth, preterm birth and SIDS). The benefits of quitting at any stage should be emphasised.

18.3 The RD&E offers an opt-out referral system to the NHS Devon stop smoking service. The midwife should refer all women who smoke or have stopped smoking in the last 2 weeks. Referral at booking is done at the time of booking the FTSC appointment. At any other time referrals can be made by calling 01884 836024. The action taken

should be recorded in the electronic patient record and follow up progress on two subsequent visits.

18.4 The midwife should use any appointment to offer referral to smoke stop service and provide support and advice.

18.5 Women will be contacted directly by the NHS Devon smoke stop service within 3 working days from referral receipt

18.6 Monitor smoking status and offer smoking cessation advice, encouragement and support throughout the pregnancy and beyond. If a woman declines referral to smoking cessation services but expresses a wish to use nicotine replacement therapy (NRT) then she should be referred to her GP for a discussion of the risks and benefits. The GP may issue a prescription following this.

18.7 The direct effects of **cannabis** on the fetus are uncertain but may be harmful. Cannabis use is associated with smoking, which is known to be harmful; therefore, women should be discouraged from using cannabis during pregnancy.

18.8 All pregnant women should have a repeat CO test performed at 36 weeks. The results discussed and documented in the electronic patient record. This data provides our Smoking at Time of Delivery data (SATOD) and is reported nationally.

19.0 AIR TRAVEL

19.1 Pregnant women should be informed that long-haul air travel is associated with an increased risk of venous thrombosis, although whether or not there is additional risk during pregnancy is unclear. In the general population, wearing correctly fitted compression stockings is effective at reducing the risk.

19.2 Pregnant women should be informed that, if they are planning to travel abroad, they should discuss considerations such as flying, vaccinations and travel insurance with their midwife or doctor.

20.0 CAR TRAVEL

20.1 Pregnant women should be informed about the correct use of seatbelts (that is, three-point seatbelts ‘above and below the bump, not over it’).

21.0 NUTRITIONAL ADVICE

Women should be offered nutritional advice in accordance with the latest, evidence-based advice available from the NHS website.

21.1 All women should be informed at the booking appointment about the importance for their own and their baby’s health of maintaining adequate vitamin D stores during pregnancy and whilst breastfeeding. In order to achieve this, women may choose to take 10 micrograms of vitamin D per day, as found in the Healthy Start multivitamin supplement.

21.2 Particular care should be taken to enquire as to whether women at greatest risk are following advice to take this daily supplement. These include:

- women of South Asian, African, Caribbean or Middle Eastern family origin
- women who have limited exposure to sunlight, such as women who are predominantly housebound, or usually remain covered when outdoors

- women who eat a diet particularly low in vitamin D, such as women who consume no oily fish, eggs, meat, vitamin D-fortified margarine or breakfast cereal
- women with a pre-pregnancy body mass index above 30 kg/m².

22.0 MANAGEMENT OF COMMON SYMPTOMS OF PREGNANCY

22.1 Women should be informed that most cases of nausea and vomiting in pregnancy will resolve spontaneously within 16 to 20 weeks and that nausea and vomiting are not usually associated with a poor pregnancy outcome. If a woman requests or would like to consider treatment, the following interventions appear to be effective in reducing symptoms:

- non-pharmacological:
  - ginger
  - P6 (wrist) acupressure
- pharmacological:
  - antihistamines.

Information about all forms of self-help and non-pharmacological treatments should be made available for pregnant women who have nausea and vomiting.

22.2 Women who present with symptoms of heartburn in pregnancy should be offered information regarding lifestyle and diet modification. Antacids may be offered to women whose heartburn remains troublesome despite lifestyle and diet modification.

22.3 Women who present with constipation in pregnancy should be offered information regarding diet modification, such as bran or wheat fibre supplementation.

22.4 In the absence of evidence of the effectiveness of treatments for haemorrhoids in pregnancy, women should be offered information concerning diet modification. If clinical symptoms remain troublesome, standard haemorrhoid creams should be considered.

22.5 Women should be informed that varicose veins are a common symptom of pregnancy that will not cause harm and that compression stockings can improve the symptoms but will not prevent varicose veins from emerging.

22.6 Women should be informed that an increase in vaginal discharge is a common physiological change that occurs during pregnancy. If it is associated with itch, soreness, offensive smell or pain on passing urine there may be an infective cause and investigation should be considered.

22.7 A 1-week course of a topical imidazole is an effective treatment and should be considered for vaginal candidiasis infections in pregnant women.

22.8 The effectiveness and safety of oral treatments for vaginal candidiasis in pregnancy are uncertain and these treatments should not be offered.

22.9 Women should be informed that exercising in water, massage therapy and group or individual back care classes might help to ease backache during pregnancy.

23.0 ASSOCIATED CLINICAL GUIDELINES/POLICIES

Clinical Guideline for Management of Women with a High BMI

Guidelines for follow up of non-attendees for antenatal care

Clinical Guideline for 11+2 – 14+1-week ultrasound/combined screen scan

24.0 MONITORING COMPLIANCE WITH THIS GUIDELINE

24.1 Any concern or non-compliance with this guideline that is identified through the investigation of clinical incidents, claims or complaints will be reviewed as per the Trust Policies regarding Incidents, Claims and Complaints, and may result in an audit and/or amendment to the guideline.

24.2 Relevant Policies:

- Incident reporting policy and procedure
- Claims management policy and procedure
- Policy and Procedure for the Management of Complaints, Concerns, Comments and Compliments

25.0 REFERENCES

NICE Clinical Guideline Antenatal care for uncomplicated pregnancies March 2008 (updated February 2019).

26.0 PUBLICATION DETAILS

Author of Clinical Guideline	Senior Matron Community / Clinics
Directorate/Department responsible for Clinical Guideline	Specialist Services/ CWH/Maternity
Contact details	6681
Version number	2.12
Replaces version number	2.11
Date written	September 2007
Approving body and date approved	Maternity Guideline Forum chairs action 09/05/2024
Review date	01/01/2027
Expiry date	09/05/2027
Date document becomes live	09/05/2024

Positive pregnancy test

Community midwife
- Should see at least once **before** 12 weeks
- Twice, if arranging first trimester screening clinic (FTSC) appointment & performing risk assessment

- Midwife & GP liaise regarding any health or social care issues
- GP may refer to midwife for:
  - Counselling re: screening
  - Risk assessment

GP

Perform risk assessment before 12 weeks (RD&E / NICE guidelines)
- Community m/w may seek GP opinion if unsure
- If GP assesses, community m/w to be informed as soon as possible < 12 weeks

Low risk = community care

- **Place order for FTSC and screening tests, via electronic patient record. Complete booking appointment prior to FTSC.**

Risk factors = refer consultant
(Or specific patient request for consultant clinic)

- **Place order for FTSC and screening tests.**
- Make referral to Obstetrics via electronic patient record.

Pre-requisites for booking FTSC appointment:
- Screening tests have been fully discussed
- Written information has been provided

Booking information complete on Electronic Patient Record
- It is imperative that all of the above are completed to facilitate the smooth running of the FTSC
- Ideal scan timing is 12-13 weeks by LMP to minimise need for rescans
- Booking should be undertaken at least 2 weeks prior to ideal scan time

N.B. The Exeter Screening Coordinators/ Fetal and Maternal Assessment Unit will continue to take responsibility for the counselling of women screened as high risk for Down's syndrome. Abnormal antenatal screening blood tests or +ve MSSU will be conveyed to community m/w or GP for further action if community booked and the hospital consultant if consultant-led care.